3.13 Acute Respiratory Failure*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Confused; does not answer questions **Objective** • PaO_2 50 mmHg, $PaCO_2$ 65 mmHg, pH 7.24 • RR 8/min, shallow	**ACTUAL** Impaired spontaneous ventilation related to respiratory muscle fatigue as evidenced by hypercapnia, acidosis and bradypnea.	• Ventilation adequate and ABG improving within 2 hours.	• Call for urgent medical help. • Open the airway and ventilate with a bag-valve-mask if needed. • Prepare for non-invasive ventilation or intubation. • Follow ABG, SpO_2, HR and consciousness.	• Achieved as pH and $PaCO_2$ improve.
Subjective • Family say, “He is not making sense.” **Objective** • GCS 10 • Restless and disoriented	**ACTUAL** Acute confusion related to hypoxemia and hypercapnia as evidenced by disorientation and reduced GCS.	• Becomes oriented as gas exchange improves.	• Check GCS and orientation hourly. • Keep the bed low and use padded rails per policy. • Reorient him often, in simple words. • Correct hypoxemia and hypercapnia as prescribed.	• Outcome reached once he is oriented.
Subjective *No specific complaint* **Objective** • Reduced consciousness • Weak cough	**RISK** Risk for aspiration as evidenced by risk factors: decreased level of consciousness and impaired cough.	• Airway stays clear.	• Raise the head of the bed to 30–45 degrees. • Keep suction ready. • Nil by mouth until swallowing is safe.	• Considered met if aspiration does not occur.

* **References for this care plan:** [35], [37], [106], [128]. Full entries are listed in the References section of the CPMN book.